Cobimetinib + vemurafenib — advanced melanoma

The treatment this guide covers Only cobimetinib has the planned days 22–28 break. Vemurafenib continues. ECG measures rhythm; an echocardiogram or other LVEF test measures pumping function, and one does not replace the other. Radiation treatment needs a separate coordinated plan.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cobimetinib + vemurafenib — advanced melanoma · support plan

Active medicines: Cobimetinib + Vemurafenib

Emetogenic risk: Cobimetinib minimal to low; vemurafenib minimal to low in NCCP 00373 v3; local policy

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- NCCP lists each component as minimal-to-low emetogenic risk; follow local and individual anti-sickness policy. Agree a diarrhoea, fluid-intake and nausea plan; contact the team if symptoms persist or you cannot drink. Check all prescribed, over-the-counter and herbal medicines for interactions before taking them. The seven-day cobimetinib break is planned; extra breaks or restarting doses require the team's instructions.
- Blood count, kidney/liver tests and LDH before each cycle. Current Cotellic product information adds CPK and creatinine monthly, and LVEF after the first month then at least every three months until treatment stops. ECG is every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Arrange monthly skin checks under current vemurafenib product information. New visual symptoms need prompt eye assessment; symptoms can require extra tests between scheduled checks.
- Contact the team promptly for new blurred or lost vision, eye pain, breathlessness or ankle swelling, palpitations or fainting, severe muscle pain or weakness with dark urine, unusual bleeding, severe diarrhoea, a blistering rash or yellow skin or eyes.
- The cobimetinib source specifies two effective contraception methods during treatment and for at least three months afterwards; vemurafenib requires at least six months after its last dose. The combined plan therefore needs to cover the longer six-month vemurafenib interval and account for reduced hormonal-contraceptive effectiveness. Discuss fertility and breastfeeding before treatment.
- Only cobimetinib has the planned days 22–28 break. Vemurafenib continues. ECG measures rhythm; an echocardiogram or other LVEF test measures pumping function, and one does not replace the other. Radiation treatment needs a separate coordinated plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00373 v3 · NCCP 00373 v3 supportive-care section; current product information for medicine-specific precautions

Sources checked 2026-11-06 · SACTCheck v0.84.42 · Regimen-level drafts; prescriber and oncology-pharmacy verification required